Malassezia (Pityrosporum) Folliculitis

Lesions typically appear on the back, chest, and sometimes the extremities in adolescents. Pruritus is more common than with typical tinea versicolor. The primary lesion is a perifollicular, erythematous, 2- to 3-mm papule or pustule.

Only appropriate culture and KOH examination can distinguish this infection from bacterial folliculitis. Frequently, biopsy with special stains for fungus is necessary, but *Malassezia* can often be recognized in follicles even in hematoxylin and eosin-stained sections.

Predisposing factors include: - Diabetes mellitus - Glucocorticoid use - Antibiotic therapy - Immunosuppressive medications

There have also been cases of recalcitrant acne vulgaris coexisting with *Pityrosporum* folliculitis.

Box 189-5: Differential Diagnosis of Pityrosporum Folliculitis

Most Likely - Bacterial folliculitis - Acne vulgaris

Consider - Candidal folliculitis - Eosinophilic folliculitis

Always Rule Out - Disseminated candidiasis - Pustular drug eruption

Inverse Tinea Versicolor

Lesions are predominantly found in flexural areas. They present as sharply demarcated, confluent, erythematous patches that may be confused with: -

Inverse psoriasis - Seborrheic dermatitis - Erythrasma - Candidiasis - Dermatophytic infections

Clinical Features of Cutaneous Variants

- Equal sex distribution
- Tend to flare during summer months (due to increased heat and humidity)
- More common in tropical climates
- Favors late adolescence and early adulthood (when sebaceous glands are most active)

Laboratory Findings

To identify *Malassezia* yeast forms: - Skin scales should be scraped onto a glass slide and treated with 10% KOH - Alternatively, use cellophane tape to collect scales; mount on a slide with methylene blue for selective staining

Microscopic appearance is classically described as "spaghetti and meatballs."

Pathology

Tinea Versicolor - Organisms inhabit the stratum corneum - Usually no dermal infiltrate - Yeast may be visible on hematoxylin and eosin staining; PAS staining is confirmatory - Microscopy shows clusters of oval budding yeast cells (3.5×4.5 µm) with short, septate, occasionally branching hyphae

Pityrosporum Folliculitis - Organisms present in widened follicular ostia mixed with keratinous material - Follicular wall rupture may lead to a mixed inflammatory and foreign-body giant cell response

Box 189-4: Differential Diagnosis of Papulosquamous Tinea Versicolor

Most Likely - Pityriasis alba - Pityriasis rosea - Seborrheic dermatitis - Dermatophyte infections

Consider - Vitiligo - Psoriasis - Pityriasis rubra pilaris - Confluent and reticulated papillomatosis of Gougerot and Carteaud

Always Rule Out - Secondary syphilis